

An Older Autistic Adult You Support at Home

For home-care workers, personal assistants & community support staff — supporting an older autistic adult (roughly 60+).

The one idea

Autism, through **monotropism**, means an older adult's attention has always locked into a few deep, intense "tunnels." Many older autistic adults were never identified and carry a lifetime of masking and unmet need. Your visits may be the most predictable, accepting relationship in their week. Protect lifelong routines, interests and objects; lower the sensory load; communicate literally; and watch for the burnout and pain that "not complaining" can hide.

What you may see — and what's really going on

You may see...	What's often happening
Same routine, objects, interests — every visit	Predictability and flow are regulation and identity
Avoids care, cancels, "doesn't like you coming"	Lifelong negative experiences + sensory/communication barriers
Slow, unusual responses; seems confused	Often processing time + autistic difference — not necessarily dementia
Doesn't report pain or symptoms	Interoception differences; "not complaining" ≠ "not suffering"
Withdrawn, exhausted	Possibly decades of accumulated burnout

Try this

- **Be the predictable, accepting presence** — same person, same time, same plan; warn before any change.
- **Protect lifelong routines, interests and objects** — removing them "for safety" can cause real harm.
- **Communicate clearly, literally and slowly**, in writing where it helps; allow ample processing time.
- **Lower the sensory load** and account for hearing/vision changes compounding lifelong differences.
- **Ask proactively about pain and symptoms** — don't wait for them to mention it.

Avoid this

- Disrupting routines or removing familiar objects.
- Assuming slowness or a different style means cognitive decline — assess, don't assume.
- Equating quiet or a "settled" presentation with being "well."

When to get more support

Watch for **autistic burnout** (often decades in the making) and **social isolation** (a major risk). The **dementia-or-difference?** question is hard and under-researched — flag changes supportively and seek autism-informed assessment rather than assuming. Report any new distress as a possible medical red flag.

If the person is a woman

Older autistic women are an especially under-identified group, having masked for a lifetime; a history of “anxiety” or burnout alongside lifelong difference warrants serious consideration of autism.

About this guide

*AI-assisted, derived from this project's research drafts — the **Family guide** (Part 5), the **Lifespan Practitioner & Health-Care guide** (Part 3.5) and the **Monotropism** brief. Uses identity-first language. **Informational — not a diagnostic tool**. Old-age evidence is thin; fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Klein et al. (2024, aging & autism); Raymaker et al. (2020); Kelly Mahler (2024); OAR (aging on the spectrum). Full citations are in the source drafts.